



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هبه عليوه علي عليوه
المحافظة : القاهرة الحي / المدينة : عابدين
رقم البطاقة : 28707140100121 رقم التليفون : 01127234857
العنوان : عابدين
العمر (تاريخ الميلاد) : 14-07-1987 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المدونة في هذا النموذج صحيحة.
رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: هبة عليوه علي عليه
National ID: 28707140100121



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

شهادة زواج
علاء محمد الحبيب



في
عنوانه على عنوانه
١٩ من شهر على حسن من مفرس
بولاية الكور - الجيزة



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٢٠٢٤/١٢ ٢٨٧٠٧١٤٠١٠٠١٢١
ربة منزل
التي
أحمد محمد حسن
البطانة سارية حتى ٢٠٣١/١٢/٢١



6/2/26, 2:45 PM

جوسناف
روسي



code:02 -- Name_A MDT -- Name_E MDT

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

صحة
EGYPT

MDT FORM

05/21/2026

Name:	Heba Eliwa Ali	MRN:	
Age:	39 Y	ID:	3510
Menopausal Sta:	premenop	Phone:	28767140100121
NRS SCORE:		Marital Status:	01115953632
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	20

Clinical Data:

BX:

ER:

PR:

HER2:

KI67:

R

Sonomammography:

Mets Workup:

History:

Premenopausal rt breast cancer since 1/2024, ER 6/8 PR 7/8 HER2 3+ Ki67 15%; initially T4c N1 received NACT AC TXT + 2C TXT/CARBO + 4HP, 9/2024 RT MRM PPR ypT3 ypN0/13, 11/2024 2 cycles kadcyla then in 3/2025 dropped out and didn't receive PORT (personal reasons). 7/2025 developed skin nodules on MRM bed - chest wall; CT CAP 7/2025 mild diff increase of skin and SC tissue of left breast, innumerable sc nodules over right br operative bed, largest nodule 21 x 16 mm, bil sus axillary LNs largest 16x13 mm, - bx from rt skin nodules TLC ER 6/8 PR 3/8 HER2 3+ Ki67 50% -- MDT 8/2025 > tykerb + xeloda

CT 2/2026 > bilateral breast masses, left breast 83x52 mm, rt breast multiple masses coalesced together forming large mass 17x11x3.9 cm, seen inseparable from underlying chest wall and muscles, bil axillary LN, retrosternal/int mammary LN

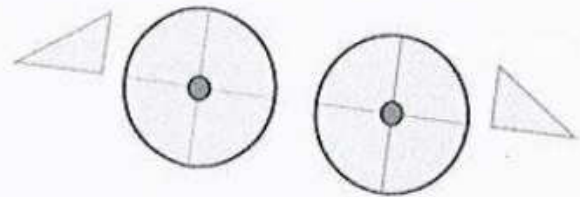
Upon clinical exam there is marked clinical progression with huge left breast mass and large bil axillary LNs + rt breast discharge

Recommendations:

for MDT as per Dr Hanna Ezzat - (stop xeloda, switch to navelbine and continue tykerb)

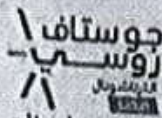
MDT Outcome & Plan

Shift to Navelbine + Tykerb with regular Clinical assessment



TNM

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient's Name:

heba elewa ali elewa

Date:

2026-04-02

Referred by:

Prof.Dr. ct cap iv

Hospital:

Goustave Roussey International Egypt

- ❖ Bilateral breast masses with enlarged axillary and internal mammary lymph nodes.
- ❖ No hepatic or pulmonary metastatic lesions

Dr. Asser Ahmed

Dr. Asser Ahmed

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- o Post right mastectomy status
- o Bilateral breast masses, on the left side it measures 83 x 52 mm on the right side showed multiple breast masses that are seen coalesced together forming large mass measuring 17 x 11 x 3.9 cm and seen inseparable from the underlying chest wall and muscles, all lesions showed heterogenous enhancement.
- o Bilateral axillary enlarged lymph nodes the largest seen on the right side measuring 36 x 33 mm showed marginal enhancement and central breaking down.
- o Retrosternal right internal mammary enlarged lymph nodes, the largest measures 16 x 14 mm
- o No pulmonary nodules, masses or cavitory lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:



Patient's Name:

heba elewa ali elewa

Date:

2026-04-02

Referred by:

Prof.Dr. ct cap iv

Hospital:

Goustave Roussey International Egypt

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Dr. Asser Ahmed

Dr. Asser Ahmed

Patient's Name: heba elewa ali elewa
Date: 2026-04-02
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

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- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

MDT FORM 20/8/2025 G

Name	هبة عليوة عاي	MRN	4606/2023
Age	37 yrs	ID	28707140100121
Menopausal status	premenopausal	Phone	01127243857
NRS SCORE		Marital status	widow
BMI		Family history	-ve
CO morbidity		REASON OF MDT	Recurrence

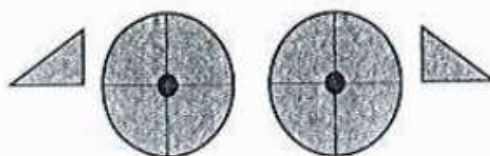
Clinical Data

BX: Rt breast ca IDCII

R

L

ER: 6/8 PR: 7/8 HER2: 43 Ki67: 15%
IHC 92 3/8 +3 50%



Sonomammography:

Mets Workup:

CT CAP 27/7 mild diffuse skin, s.c tissue
+ left breast.
bilateral axillary LN
Evidence of Rt mastectomy & innumerable s.c
nodules at op. wound bed, Rt chest wall & incision
skin thickness

1/24

TNM

initially T1c N1

y pT3 y pNc

Recommendations: premenopausal Rt breast Ca since 1/2024
Received NACT 4A + 4 Taxotere + 2 cycle TXT + Carbo + HPX4
(12/9/2024) MRM y pT3 / pNc 19/11/2024
received 2 cycle Kadcyla (26/3/25) then Dropout for 3 Month and
didn't receive PORT. 11/7/2023 skin nodules on MRM Scan and chest wall
biopsy from recurrent nodules and New CTCAP was done.
she on Hormonal treat till now For MDT

MDT OUTCOME & PLAN

ER 6/8 PR 3/8
HER2+3 Ki67 50%

C CO / N / 11

for start Tykerb + Xeloda

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name :	الأستاذة / هبة عليوة عليوة	Request No. :	13008257
Patient No. :	28707140100121	Request Date :	28/07/2025
Path No. :	2383/25	Verification Date :	02/08/2025
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	38 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

Clinical Data.

Right breast mastectomy , ?recurrence (BIRADS4)

Nature of specimen

Tru cut biopsy collected on 27/7/2025.

Gross examination

Three grey white tissue cores ranging from 0.5 up to 2 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Recurrent Invasive classic lobular carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- * DCIS: Absent
- * Lobular intra-epithelial neoplasia : Absent.
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: Absent
- * Microcalcifications : Absent
- * IPT (Positive control showed appropriate reactivity) :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: Weak nuclear reaction in 5 % tumor cells (Allred score 3/8)
- * HER2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- * Ki 67: 50 %

Diagnosis

Right breast mass, tru cut biopsy:

Verified by :

Dr.Heba Hasan

Printing Date : 10/08/2025 08:49:13

Page 1 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

الاستاذ / هبة عليو عليو

:28707140100121

:2383/25

:Female

:38 Y

ng Doc : *

Request No. : 13008257

Request Date : 28/07/2025

Verification Date : 02/08/2025

Financial Category : جوستاف روبي - نفقة الدولة

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Recurrent Invasive classic lobular carcinoma, Grade 2 .

IPT:

- T
- * ER: Negative (score 0/8).
 - * PR: Positive (score 3/8).
 - * HER2: Positive (score 3+).
 - * Ki67: 50 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

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Page 2 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / هبة عليود علي عليوه	Request No. : 13001284
Patient No. : 28707140100121	Request Date : 24/10/2024
Path No. : 4081/24	Verification Date : 06/11/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Maximal tumor diameter: 9x7 cm.
- Actual tumor cellularity: 15%, tumor necrosis index: 85%.
- Free resection Margins.
- Free nipple & areola.
- Lymph nodes: 0/13, some showed therapy induced changes.
- Partial pathological response to therapy
- Stage: ypT3, ypN0.
- RCB: 2.118, RCB class II.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
DR.Menna Nabil

Printing Date : 12/11/2024 11:41:36

Page 2 of 2

Printing By : .marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Heba Elewa Ali Elewa	Date : 27/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 38 Year(s)
Medical ID:	Pat. ID: 46003

Patient Name: Heba Elewa Ali Elewa

Date: 29-7-2025

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

FINDINGS:

- A small subpleural nodule is seen at the lateral segment of the middle lung lobe measuring about 3.5 mm
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses or consolidative patches
- Normal C.T. appearance of the mediastinum vessels.
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- Mild diffuse increase in skin and subcutaneous tissue of the left breast.
- Bilateral suspicious axillary lymph nodes, largest measuring 18 x 13 mm
- Evidence of right mastectomy with innumerable subcutaneous nodules are seen at the operative bed as well as the right axilla and the right lateral chest wall ,largest measuring about 21 x 16 mm with mild diffuse increase in skin thickness.
- Mild dorsal scoliosis with leftwards concavity.



CONCLUSION :-

- A small subpleural middle lobar nodule
- Mild diffuse increase in skin and subcutaneous tissue of the left breast.
- Bilateral suspicious axillary lymph nodes
- Evidence of right mastectomy with innumerable subcutaneous nodules as formentioned .For correlation with sonomammography
- Mild dorsal scoliosis with leftwards concavity

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS:-

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

HISTORY :As compare to previous report dated 11/01/2024,the current study reveals:

FINDINGS:

- Normal C.T. density of the liver parenchyma, with no focal lesions .
- No extra or intra-hepatic biliary dilatation .
- Both kidneys are functioning and normal in dimensions .**Still noted** left simple cortical renal cyst measuring 5.3 cm.
- The spleen and pancreas show normal C.T. appearance .
- Normal size and C.T. density of the adrenal glands .
- No ascites .
- The urinary bladder shows normal size and wall thickness with no calculi inside.
- The uterus is of normal C.T. appearance .
- Normal ischio-rectal fossae on both sides .
- No enlarged abdominal or pelvic lymph glands.



IMPRESSION:

Unremarkable study apart from left simple cortical renal cyst

Much Obligated,

Mona Ali,MD



Women's Health Outreach Program

Name: heba elioa aly elioa

Date: 14/11/2023

Bilateral Full field Digital Mammography

Clinical summary: right breast palpable lump.

Technique

- Mediolateral oblique and CC views.
- High resolution ultrasound.

Detailed Findings

Right breast

- Homogenous fibroglandular pattern (ACR b).
- UIO ill-defined asymmetrical density about 7.5 x4.5 cm with suspicious microcalcifications within it ..with skin thickening
- Ultrasound examination confirmed the hypoechoic solid lesion seen located from 12-4 o'clock with mild interstitial edema .the lesion reaching to nipple away' from skin by 1cm
- Multiple pathological lymph nodes .largest at level I measures 17x11mm and level II measures 11x6mm and at infra clavicular measures 6x4mm

left breast

- Homogenous fibroglandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Right breast: (BI-RADS 5): UOQ malignant looking mass lesion.
left Breast: (BI-RADS 1): Normal study.

Recommendation

-Ultrasound guided biopsy was done at the same settings.

Best regards,
Dr Marwa M.Hassan



Women's Health Outreach Program

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Date: 14/11/2023

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Best regards,

Dr Marwa M.Hassan

محمد عبد الحليم علي

99mTc MDP Bone Scan Report (25/5/2024)

Whole Body Anterior and Posterior Planar Views Acquired 20hrs. Post I.V.
Injection of 20mCi of ^{99m}Tc MDP. Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Mild degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Attn: Elsayed

Mohamed Abdelhameed, MSc

د. محمد عليوة علي

99mTc MDP Bone Scan Report (25/5/2024)

Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V.
Injection of 20mCi of ^{99m}Tc MDP. **Revealed:**

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Much Oblige!

Attn: Elzayed

Mohamed Abdelkawab, MSc



13/05/2020

Patient's Name:

Heba Aliwa Ali Aliwa

Date:

2026-08-20

Referred by:

Prof.Dr. no doctor

Hospital:

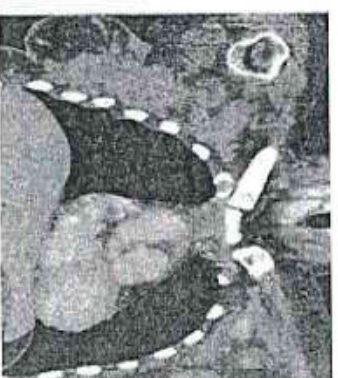
Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

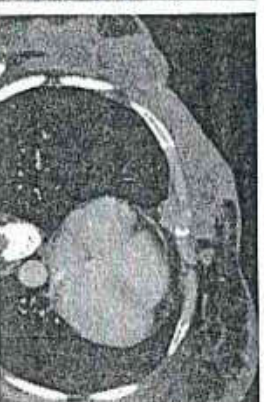
History: known patient with breast cancer on CTH. No available previous studies for comparison at time of reporting.

I-CHEST:

- The right MRM operative bed shows multiple amalgamated soft tissue masses infiltrating the underlying chest wall and overlying skin, extending along the lateral aspect to reach the axilla. Multiple lesions also extend laterally and posteriorly till the upper abdominal region.



- Multiple left breast soft tissue masses infiltrating the skin, which is diffusely thickened. Associated multiple pathological axillary, inter and retropectoral LNs.



- Multiple pathological LNs are seen involving the anterior mediastinum, right internal mammary group (a discrete one is seen on the left) and cardiophrenic groups.
- Multiple subcentimetric bilateral supraclavical LNs, of average 6mm.
- Few tiny bilateral pulmonary nodules of average 2mm.
- Bilateral lower lobe basal thick fibroatelectatic bands.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size of the heart. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.

Patient's Name:

Heba Aliwa Ali Aliwa

Date:

2026-08-20

Referred by:

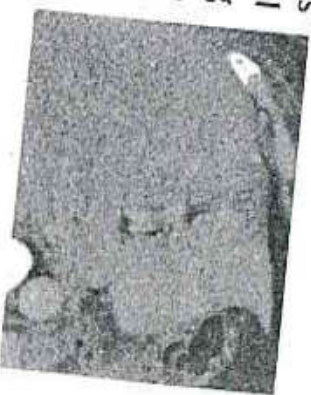
Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

II-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous parenchymal attenuation. Ill-defined geographical hypodense lesion implicating the region surrounding the falciform ligament likely focal fatty infiltration. No sizable suspicious focal lesions. No dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- **Calcular gall bladder.**
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No definite masses or back pressure changes. Left **upper polar simple cyst measuring 3.2x4cm.**
- Normal CT appearance of the spleen, adrenal glands and pancreas.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.



- Normal appearance of the pelvic organs with no collections.
- No sizable suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- The large bowel diffusely shows submucosal hypodensity likely fat attenuation suggestive of chronic inflammation sequelae.
- Right adnexal cystic lesion measuring 12x17mm likely functional.
- Bone window reveals sclerotic lesion at the lateral aspect of left 4th rib.

OPINION :

- Known history of breast cancer, current study reveals:
- Diffuse infiltrative masses in the right MRM operative bed with extensions as detailed.
- Left breast soft tissue masses and pathological axillary LNs.
- Anterior mediastinal, internal mammary and cardiophrenic pathological LNs.
- Few tiny bilateral pulmonary nodules.
- Left 4th rib sclerotic lesion for bone scan correlation.

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